

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in New York

A Licensed Home Care Services Agency (LHCSA) — private pay & Medicaid (MLTC).

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The New York model — read this first

In New York, an agency that sends aides into people's homes to provide hands-on personal care must be a Licensed Home Care Services Agency (LHCSA), licensed by the New York State Department of Health (DOH) under Public Health Law Article 36. New York is one of the hardest states in the country to enter — not because the license is complex, but because DOH applies a “public need” test that effectively closes new LHCSA licensure in the big markets (all five NYC boroughs, Nassau, Suffolk, Westchester). This guide walks the whole path honestly — including the two realistic ways in (buy an existing LHCSA, or run a truly companion-only business) — broken down so a first-timer can follow it.

Read this before you spend a dollar — the New York entry barrier. There's no blanket moratorium in 2026, but DOH presumes there is NO public need for a new LHCSA in any county already served by five or more LHCSAs — which describes every high-demand market in the state. Overcoming that presumption is a heavy burden. So the two realistic paths are: (1) BUY an existing LHCSA — a change of ownership of an agency already serving 25+ patients bypasses the public-need review; or (2) run a truly COMPANION-ONLY business (no hands-on personal care), which may not need an LHCSA at all. This guide covers all three, but be clear-eyed: de novo LHCSA licensure in NYC/Long Island/Westchester is effectively closed.

And budget for the LLC publication cost. New York is one of the few states that makes a new LLC publish a formation notice in two county-designated newspapers for six weeks — which can cost several hundred dollars upstate and well over a thousand (sometimes several thousand) in Manhattan. It's a real, often-surprising startup cost (Part 3).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with DOH and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide your path	De novo LHCSA, buy an existing LHCSA, or companion-only (Parts 1-2)
2	Form the business & insure	Entity, EIN, NPI, insurance — and the NY publication step (Part 3)
3	Get the license (or buy one)	LHCSA application or change-of-ownership; the RN requirement (Part 4)
4	Build your clinical leadership	An Administrator + an RN Director of Patient Services (Part 4)
5	Recruit & train aides	PCA 40-hour / HHA 76-hour training + the registry (Part 5)
6	Set up EVV & operations	eMedNY EVV, policies, wage-parity compliance (Part 6)

7	Serve private-pay clients	Start earning on the license (Part 8)
8	Contract for Medicaid (MLTC)	Contract with Managed Long Term Care plans (Part 8)
9	Get your first clients	Discharge planners, the 59 AAAs, NY Connects (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. *Decide your path FIRST — in a saturated NY market, buying an existing LHCSA or running companion-only is usually more realistic than a new license. Whichever path, line up your RN Director of Patient Services early (an LHCSA can't operate without one) and budget the publication cost.*

PART 1

What This Business Is

In short: A Licensed Home Care Services Agency — you employ and send aides to provide hands-on personal care in clients' homes. The license is required for personal care; companion-only work may not need it.

New York regulates agencies that provide “home care services” — personal care aide (PCA) and home health aide (HHA) services, and nursing — as LHCSAs under PHL Article 36 (regulations at 10 NYCRR Parts 765 and 766). Hands-on help with the activities of daily living (bathing, dressing, toileting, transferring, feeding, mobility) is “personal care” and requires the LHCSA license. Pure companionship, homemaking, and sitter services with NO hands-on personal care may fall outside the license — but the line is easy to cross.

Your three possible paths

Path	What it is	Reality in NY
New LHCSA license	Apply to DOH for a fresh license	Blocked by “public need” in saturated counties
Buy an existing LHCSA	Change of ownership of an active agency	The standard way into saturated markets (bypasses public need if 25+ patients)
Companion-only business	No hands-on personal care	May not need an LHCSA — but the scope line is strict

Key terms. LHCSA = Licensed Home Care Services Agency (your license). DOH = NY State Dept. of Health. PHHPC = the council that decides public need. PCA/HHA = Personal Care Aide / Home Health Aide. MLTC = Managed Long Term Care (how most Medicaid home care is delivered). CDPAP = Consumer Directed Personal Assistance Program (now a single fiscal intermediary, PPL). Wage Parity = the NYC-area home-care wage/benefit floor. eMedNY = NY Medicaid's system (and EVV). NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. New York is the largest, highest-cost home-care market in the country — home to the biggest Medicaid long-term-care program in the nation and a rapidly aging population — which means strong private-pay rates and enormous, durable demand for anyone who gets in. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Your Path In — Public Need, Buying, or Companion-Only

In short: *The single most important New York decision is how you get in. Here are the three paths and when each makes sense.*

Path 1 — A new LHCSA license (hard in saturated markets)

DOH reviews a new LHCSA application on four factors, including public need. It presumes no public need in any county already served by five or more LHCSAs — which covers the entire NYC metro and most populous counties. You can try to rebut the presumption with evidence (documented waiting lists, cultural/linguistic or specialty gaps, quality problems at incumbents), but it's a heavy burden decided by the Public Health and Health Planning Council. Realistic mainly in genuinely underserved (often rural) counties.

Path 2 — Buy an existing LHCSA (the common way in)

A change-of-ownership application for an LHCSA that is actively serving 25 or more patients bypasses the public-need review entirely — it's evaluated only on financial feasibility and the character/competence of the new owners. This is how most operators enter saturated New York markets. Budget for the purchase price and do careful due diligence (compliance history, contracts, liabilities) with counsel.

Path 3 — A companion-only business (no license, strict line)

A business that provides only non-hands-on companionship, homemaking, and sitter services may operate without an LHCSA. The moment your staff help with bathing, transferring, toileting, or other personal care, you need the license. If you take this path, put the scope in writing for clients and staff, and confirm the permissible-task boundary with counsel — regulators take it seriously.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In New York you form the LLC through the Department of State, Division of Corporations (dos.ny.gov) — the filing fee is \$200 — and then complete New York’s distinctive newspaper publication requirement, which most other states don’t have. You’ll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization.** With the NY Department of State, Division of Corporations at dos.ny.gov — the filing fee is \$200.
- 2. Name a registered agent and adopt an Operating Agreement.** In New York the Department of State is the default agent for service of process, but you still designate an address to forward legal mail; the Operating Agreement — required by NY LLC law — sets ownership and management.
- 3. Complete the newspaper publication (the NY step).** Within 120 days of formation, publish a formation notice for six consecutive weeks in TWO newspapers designated by the County Clerk where your office sits — you don’t get to pick the papers — then file a Certificate of Publication with DOS (\$50).
- 4. Calendar your Biennial Statement.** \$9 every two years to the Department of State keeps the LLC in good standing.

Budget for the New York publication requirement. *New York is one of the only states that makes a new LLC publish a formation notice in two county-designated newspapers for six consecutive weeks. The newspaper charges typically run a few hundred dollars upstate but can exceed \$1,000–\$2,000 in NYC (especially Manhattan) — a real, often-surprising startup cost. Miss the 120-day window and DOS can suspend your authority to do business in New York, so calendar it the day you form.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business’s federal tax number — think of it as a Social Security number for the company. It’s free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here’s exactly how to get it:

- 1. Go to [IRS.gov](https://irs.gov) and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party’s legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can’t be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You’ll use it to enroll with New York Medicaid (eMedNY) and bill the MLTC plans. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.
- 3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.
- 4. Submit — it's free and quick.** It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of New York branches — Chase, Bank of America, Citibank, M&T, KeyBank, or TD Bank — or a New York community bank or credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so you need coverage built for that risk — and in New York, workers' compensation AND statutory disability benefits coverage are mandatory for employees:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (an aide's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by an aide — general liability EXCLUDES this	Yes — high exposure
Non-Owned & Hired Auto	Your aides driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation & Disability	Aide injuries; New York also mandates statutory disability benefits	Required in NY for employees
Employee Dishonesty Bond	Theft from a client's home — families ask if caregivers are “bonded”	Strongly recommended

New York requires disability coverage too. On top of workers' compensation, New York employers must carry statutory disability benefits (DBL) — and, since 2018, Paid Family Leave, which is added onto the DBL policy. Your payroll provider or broker sets these up; factor them into your labor cost.

Real places to get it

Buy through an independent agent or broker who writes New York home-care programs. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local New York agent, use trustedchoice.com (filter for home care), or ask a franchise or peer. Confirm the carrier writes home care in New York before you rely on a quote.

What to ask the insurance agent

“Do you write NY LHCSAs — which carriers, and what's the minimum premium?”

“Is abuse & molestation included or a sub-limit, and do you include non-owned & hired auto and an employee-dishonesty bond?”

“What coverage limits do the MLTC plans, the VA, or long-term-care insurers require?”

“Can you set up New York workers' comp AND statutory disability benefits (and Paid Family Leave)?”

PART 4

Get Your License (or Buy One) & Build Leadership

In short: Whether you apply *de novo* or buy an existing agency, you'll need an Administrator and an RN Director of Patient Services, plus DOH approval. The application fee is \$2,000.

4.1 The people an LHCSA must have

- **Administrator:** holds a baccalaureate (or equivalent experience), on-site during operating hours, responsible for day-to-day operations.
- **Director of Patient Services (RN):** a New York-licensed registered nurse with at least two years of clinical and one year of supervisory home-care experience — required by 10 NYCRR 766. Aide services must be RN-supervised, so this role isn't optional. Line up your RN early.

4.2 The application / change of ownership

1. **Choose your path (Part 2).** New license (DOH-1056a, subject to public need) OR change of ownership of an existing LHCSA (bypasses public need at 25+ patients).
2. **Pay the \$2,000 application fee.** Non-refundable, per application (PHL § 3605).
3. **Show financial feasibility.** CPA-certified working capital of at least two months of operating expenses.
4. **Pass DOH review + the initial survey.** DOH reviews character/competence (and public need for a new license); an initial on-site survey follows approval within about 90–180 days.
5. **Register annually.** Annual LHCSA registration is mandatory — you can't operate or be reimbursed without it (non-registration penalties apply).

PART 5

Your Aides — Training, Registry & Screening

In short: New York sets specific aide training hours and runs a home care registry. Build your hiring around the PCA/HHA pipeline and RN supervision.

5.1 Aide training (NY-specific hours)

- **Personal Care Aide (PCA):** a 40-hour DOH-approved training program.
- **Home Health Aide (HHA):** a 76-hour DOH-approved program (builds on the PCA curriculum plus health-related skills).
- **Approved programs only:** training must come through a DOH-approved PCA/HHA training program.

5.2 Registry, background & health

1. **Verify the aide on the Home Care Registry.** DOH's registry lists HHAs/PCAs; verify status before assigning an aide.
2. **Complete the Criminal History Record Check.** Fingerprint-based state (DCJS) and FBI checks through DOH's CHRC program before permanent employment.
3. **Do the health assessment / TB screening.** A pre-employment health assessment including TB screening (confirm the current protocol with DOH).
4. **Supervise with your RN.** Aide services are supervised by your RN Director of Patient Services per the plan of care, with periodic supervisory visits.

Mind the wage floors. New York sets a home care aide minimum wage (\$19.65/hr in NYC, Long Island, Westchester; \$18.65 upstate as of Jan 1, 2026 — verify), and NYC-area agencies must also meet the Home Care Worker Wage Parity Law (extra wage + benefit floor). Build these into your rates and your MLTC contracts.

5.3 Building your aide pipeline (the New York playbook)

The hardest ongoing job in New York home care isn't the license — it's keeping enough trained, registry-listed aides on your roster to say yes when an MLTC care manager or a discharge planner calls. Because hands-on aides must complete DOH-approved PCA (40-hour) or HHA (76-hour) training and clear the Criminal History Record Check before they can work, your pipeline IS your business. Here is how established New York operators keep it full:

1. **Recruit already-trained PCAs and HHAs first.** There are hundreds of thousands of trained aides across New York, concentrated in the NYC metro. Post on Indeed, ZipRecruiter, and local Facebook groups; run a caregiver-referral bonus (\$150–\$300 pays for itself); and always collect the aide's details so you can confirm their status on the DOH Home Care Registry before the first shift.
2. **Partner with a DOH-approved training program.** Approved PCA/HHA programs run constantly, especially downstate. A standing relationship with one or two schools gives you first look at new graduates — offer to be their placement partner and some will send you candidates before they finish the hours.
3. **Consider running your own DOH-approved training.** Once you have volume, operating an approved PCA/HHA program turns your biggest cost center into a recruiting funnel — you train aides to your own standard and hire the best. It's how many large New York agencies solve the shortage permanently.
4. **Compete on speed, hours, and respect — not just wage.** In the NYC/Long Island/Westchester wage-parity zone the base wage and benefit floor are set by law, so aides pick the agency that interviews fastest, schedules consistent hours, pays correctly and on time, and treats them well — rarely the one quoting a few cents more.

Verify on the registry before every assignment. New York holds YOU responsible for confirming an aide is trained, listed on the Home Care Registry, and CHRC-cleared before they touch a client. Make registry verification a hard step in your hiring checklist — an unverified or lapsed aide discovered during a survey or MLTC audit can cost you a contract.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Getting Operational

In short: *Your policies come from the licensure standards; add eMedNY EVV for Medicaid visits and your scheduling/billing systems, and stay compliant with wage parity.*

6.1 Policies

Build your P&P Manual to 10 NYCRR Parts 765/766: client rights, the plan of care and RN supervision, aide competency, complaint handling, incident and abuse reporting, emergency preparedness, and infection control.

6.2 Set up EVV

New York requires Electronic Visit Verification for Medicaid personal care and home health. NY uses a “choice model” integrated with eMedNY — you may use a compliant EVV vendor that submits to the state aggregator. A visit must be verified to be paid. Private-pay work isn't subject to EVV.

6.3 Systems

Pick a home-care platform for scheduling, records, visit tracking, invoicing, and payroll that supports NY EVV and wage-parity tracking.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: *A client is referred (privately or via an MLTC plan), your RN assesses and sets the plan of care, an aide delivers the hours (with EVV for Medicaid), the RN supervises, and you review and adjust.*

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a CHHA or hospice partner, or an MLTC plan (Part 10). Take the basics — who, what tasks, how many hours, where, when, and who pays (private, MLTC, VA, or LTC insurance).

Stage 2 — RN assessment & the plan of care

Your RN assesses the client and builds the plan of care that the aide follows. For MLTC clients, the plan's authorization sets the approved hours; your plan of care aligns to it.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). Assign a registry-verified, trained aide and start with an RN-supervised introduction.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The aide delivers care; Medicaid visits are captured in EVV. Protect the client-aide match — consistency drives satisfaction and retention.

Stage 5 — RN supervision & quality

Your RN makes periodic supervisory visits; you monitor performance and changing needs, run your quality program, and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As needs change, the RN updates the plan of care; for MLTC clients, coordinate reassessments and new authorizations with the plan. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any plan, and document. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid (MLTC), VA & LTC Insurance

In short: Start on private pay and long-term-care insurance; earn most Medicaid revenue by contracting with MLTC plans. New York is a huge, high-cost, heavily-regulated market.

Your revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start on your license; NY is high-cost (esp. NYC/LI)
Medicaid — via MLTC plans	Managed Long Term Care + Personal Care Services	Most Medicaid home care; you contract with MLTC plans
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 1, Optum)

8.1 Private pay

On your license you serve private-pay clients — billed hourly with a visit minimum. New York's state median is around \$34/hour for homemaker care (2024), but NYC, Long Island, and Westchester run materially higher, so metro agencies quote well above that. Set your rate above your fully-loaded (wage-parity-compliant) aide cost. Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL/cognitive triggers are met — you bill the family or the insurer.

8.2 Medicaid — MLTC (and the CDPAP change)

Most adult Medicaid home care in New York is delivered through Managed Long Term Care plans, and an LHCSA earns Medicaid revenue by contracting with MLTC plans (and subcontracting HHA hours to Certified Home Health Agencies). Note a big 2025 change: the Consumer Directed Personal Assistance Program (CDPAP) was consolidated to a SINGLE statewide fiscal intermediary — Public Partnerships LLC (PPL) — so the old CDPAP-fiscal-intermediary business model is effectively closed. New entrants pursue the LHCSA + MLTC-contract path.

1. **Be a licensed/registered LHCSA first (Part 4).**
2. **Contract with MLTC plans in your area.** Credential and negotiate rates plan-by-plan; set up EVV.
3. **Comply with wage parity (NYC area)** and the home care aide minimum wage in your rates.
4. **Bill through the plan / eMedNY** against verified EVV visits. Medicaid/MLTC rates are plan- and region-specific — pull them from your contracts, don't rely on a quoted number.

8.3 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** contract through the VA Community Care Network — New York is in Region 1, administered by Optum (provider line 888-901-7407).
- **Aid & Attendance:** an increased VA pension the veteran can apply toward your private-pay care (verify current amounts on VA.gov).

PART 9

What It Costs & How Long It Takes

In short: New York has real barriers to entry — plan for the path (buy vs. new), the RN, the publication cost, and MLTC contracting. This is a high-investment, high-opportunity market.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
Path cost	Buying an existing LHCSA (if that's your path)	Varies widely — do due diligence
LHCSA application	\$2,000 (per application)	\$2,000
RN Director of Patient Services	Required leadership hire	Ongoing salary
Working capital	2 months operating expenses (CPA-certified)	Several months of costs
Business setup + publication	LLC \$200 + publication (hundreds to \$2,000+ in NYC) + \$9 biennial	\$200 + publication + legal
Insurance	Liability + abuse + non-owned auto + dishonesty + WC/disability	Get quotes — annual
EVV & software	eMedNY-compliant EVV + scheduling/billing/payroll	Monthly SaaS
Payroll float	Wage-parity payroll before MLTC pays you	Working capital

Accreditation. New York doesn't require national accreditation for an LHCSA, but ACHC/CHAP/TJC accreditation can help with MLTC and VA contracting and signals quality. Optional; add it when volume justifies it.

9.2 Timeline

First	Choose your path	New license, buy an existing LHCSA, or companion-only
Early	RN + business + publication	Line up your RN; form the LLC and complete publication
Months	License / change of ownership	DOH review + initial survey (~90–180 days after approval)
Right away	Start private pay	Serve private-pay & LTC-insurance clients on the license
Then	Contract MLTC	Credential with MLTC plans; set up EVV & wage parity
Ongoing	Referrals & census	Discharge planners, the AAAs, NY Connects

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$34/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The line items above are the visible costs. The ones that sink underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay aides every week or two, but MLTC plans and long-term-care insurers pay you 30–60 days after you bill. On a growing Medicaid caseload you can be out tens of thousands of dollars in wages you've paid but not yet collected. Keep 8–12 weeks of payroll in reserve, or launch on private pay (which pays up front) while your Medicaid line ramps.
- **The publication surprise:** New York's newspaper publication requirement can run well over \$1,000 in the NYC metro — an easy cost to forget when you budget only the \$200 filing fee.
- **Unbillable overhead:** your own time, your scheduler/coordinator, office and phone, background checks, and the RN supervisory visits your license and MLTC contracts require — none of it is billable, all of it is real.
- **Turnover churn:** every aide who quits costs you re-recruiting, re-verifying on the registry, and re-training. Budget for it, and spend on retention instead — it's cheaper than replacement.
- **Denied and delayed claims:** a Medicaid claim that doesn't match a verified EVV visit, or misses timely filing, simply doesn't get paid. Reconcile every remittance and chase exceptions weekly — unreconciled claims are money you earned and lost.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, CHHA/hospice partners, the 59 Area Agencies on Aging, NY Connects, care managers, and senior communities. Build these before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the top referral engine for post-acute home care — make same-day starts easy.
- **CHHA & hospice partners:** Certified Home Health Agencies subcontract HHA hours to LHCSAs, and hospices need personal-care support — core reciprocal channels.
- **The aging network:** New York's 59 Area Agencies on Aging (NYSOFA) and NY Connects (the statewide “no wrong door” line) field families needing in-home help — get listed and known.
- **MLTC plan care managers:** once contracted, plan care managers route member hours to in-network agencies.
- **Geriatric care managers & senior communities:** Aging Life Care professionals, assisted living, NORCs, and adult day programs refer supplemental care.
- **NY 2-1-1:** list your services with NY 2-1-1 (dial 211).

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the path & staff

Who	Contact	What to verify / ask
NY DOH — Home Care (LHCSA)	health.ny.gov/facilities/home_care/lhcsa	The current public-need posture; new license vs. change of ownership; the \$2,000 fee; DOH-1056a
DOH — Criminal History Record Check	health.ny.gov (CHRC)	Fingerprint checks for aides
DOH-approved PCA/HHA training programs	health.ny.gov (training program list)	The 40-hour PCA / 76-hour HHA training; the Home Care Registry
NY Dept. of Labor — home care wages	dol.ny.gov	The home care aide minimum wage and Wage Parity
Health-care counsel	a NY home-care attorney	Your entry path; a change-of-ownership purchase; the companion-only scope line

Business, insurance & billing IDs

Who	Contact	What to verify / ask
NY Department of State	dos.ny.gov	Forming your LLC (\$200); the publication requirement; the \$50 Certificate of Publication; the \$9 biennial statement
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI
Insurance agent (home-care program)	trustedchoice.com; carriers in Part 3.3	Liability + abuse + non-owned auto + dishonesty + WC/disability

Funding, EVV & referrals

Who	Contact	What to verify / ask
MLTC plans	the plans serving your area	Contracting; rates; authorizations; EVV; wage-parity terms
eMedNY — EVV	emedny.org (EVV)	The choice-model EVV requirements
VA Community Care / Optum (Region 1)	888-901-7407 · va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN
NY Connects (ADRC)	nyconnects.ny.gov	Getting listed; local referral partners

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (59)	aging.ny.gov (NYSOFA)	Local information & referral for older adults
NY Connects	nyconnects.ny.gov	The statewide no-wrong-door LTSS line
NY 2-1-1	dial 211	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; MLTC/VA requirements

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to buy an existing agency, or can I get a new license?

In saturated New York markets — the five boroughs, Long Island, Westchester — a new LHCSA license is effectively blocked by the public-need test, so most new entrants either buy an existing LHCSA (which bypasses public need at 25+ patients) or run a companion-only business. In genuinely underserved counties a new license can still be viable. Decide your path before you spend a dollar.

Do I really need an RN before I open?

Yes. An LHCSA must have a Director of Patient Services who is a New York-licensed RN with home-care experience, and aide services must be RN-supervised — so line up your RN early; you cannot operate the license without one.

You can do this

Opening a New York home care agency is one of the harder entries in the country — but the market is enormous, and operators who pick the right path (usually buying an existing LHCSA or running companion-only) build real businesses. Take it one rung at a time: choose your path, line up your RN, form and publish the business, get licensed (or acquire a license), train and register your aides, launch on private pay, and contract with the MLTC plans. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in Public Health Law Article 36, 10 NYCRR Parts 765/766, and current NY Medicaid information as of 2026. This is general information, not legal, tax, or billing advice.

Requirements, fees, rates, and contacts change — verify each with DOH and qualified counsel before you rely on it. In particular, confirm the current public-need posture, the aide wage floors and Wage Parity, and your entry path before you rely on them.